TABLE 7
RELATIONSHIP BETWEEN HRR CATHETERIZATION RATE, SURVIVAL, AND COSTS, BY CLINICAL APPROPRIATENESS FOR INTENSIVE MANAGEMENT

	OLS ESTIMATES OF THE RELATIONSHIP BETWEEN HRR RISK-ADJUSTED CATH RATE AND			
SAMPLE	One-Year Survival (1)	One-Year Cost ($1,000s) (2)	Beta-Blocker in Hospital (3)	Catheterization within 30 Days (4)
A. All patients (N = 138,873)	.007	8.093	−.28	.702
	(.019)	(1.410)	(.073)	(.004)
B. By cath propensity:				
Top tercile (N = 46,287)	.052	10.012	−.366	.802
	(.019)	(1.439)	(.073)	(.032)
Middle tercile (N = 46,295)	.03	11.154	−.271	.906
	(.030)	(1.784)	(.082)	(.021)
Bottom tercile (N = 46,291)	−.075	2.763	−.209	.369
	(.028)	(1.612)	(.073)	(.021)
Difference (top − bottom)	.127	7.249	−.157	.433
	(.034)	(2.161)	(.103)	(.038)
C. By age:				
65–80 (N = 96,093)	.023	9.616	−.311	.775
	(.021)	(1.448)	(.072)	(.012)
Over 80 (N = 42,780)	−.031	4.738	−.215	.531
	(.028)	(1.603)	(.080)	(.022)
Difference (top − bottom)	.054	4.878	−.096	.244
	(.035)	(2.160)	(.108)	(.025)
D. By AHA/ACC criterion:				
Ideal (N = 89,569)	.027	9.845	−.302	.769
	(.023)	(1.599)	(.076)	(.010)
Appropriate (N = 31,800)	−.002	6.174	−.282	.752
	(.024)	(1.537)	(.080)	(.026)
Not appropriate (N = 17,504)	−.08	2.958	−.177	.264
	(.040)	(1.511)	(.065)	(.021)
Difference (top − bottom)	.107	6.887	−.125	.505
	(.046)	(2.200)	(.100)	(.023)

NOTE.—Cath propensity is an empirical measure of patient appropriateness for intensive treatments. We define this measure by using fitted values from a logit model of the receipt of cardiac catheterization on all the CCP risk adjusters. HRR surgical and medical intensity rates are computed as the risk-adjusted fixed effects from a patient-level regression of the receipt of cath or beta-blockers on HRR fixed effects and CCP risk adjusters.